

SENATE CHAMBER

STATE OF OKLAHOMA

DISPOSITION

☒ FLOOR AMENDMENT

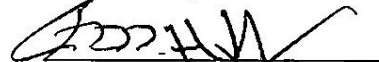
No. 1

☐ COMMITTEE AMENDMENT

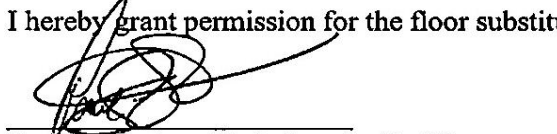
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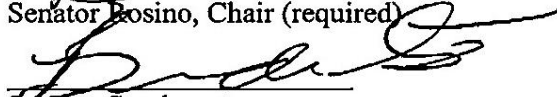
I move to amend House Bill No. 2749 by substituting the attached floor substitute (Request #3985) for the title, enacting clause and entire body of the measure.

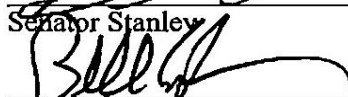
Submitted by:


Senator Hall

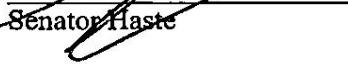
I hereby grant permission for the floor substitute to be adopted.

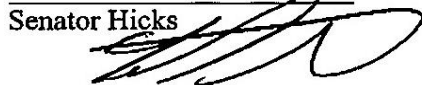

Senator Rosino, Chair (required)


Senator Stanley


Senator Coleman


Senator Dessett


Senator Haste


Senator Hicks

Senator Paxton, President Pro Tempore

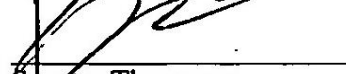
Senator McIntosh

Senator Nice

Senator Pugh


Senator Reinhardt

Senator Standridge


Senator Thompson

Senator Daniels, Majority Floor Leader

Note: Health and Human Services committee majority requires seven (7) members' signatures.

I hereby grant permission for the floor substitute to be adopted.


Senator Hall
Appropriations Committee Chair

Hall-DC-FS-HB2749
4/28/2026 2:59 PM

(Floor Amendments Only)

Date and Time Filed: 4-29-26 10:37am jd

☐ Untimely

☐ Amendment Cycle Extended

☐ Secondary Amendment

STATE OF OKLAHOMA

2nd Session of the 60th Legislature (2026)

FLOOR SUBSTITUTE
FOR ENGROSSED

HOUSE BILL NO. 2749

By: Caldwell (Trey) and Pittman
of the House

and

Hall of the Senate

FLOOR SUBSTITUTE

[state Medicaid program - nursing facility
reimbursement - add-on rate - federal approval -
rules - effective date -
emergency]

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. AMENDATORY 63 O.S. 2021, Section 1-1925.2, is
amended to read as follows:

Section 1-1925.2. A. The Oklahoma Health Care Authority shall
fully recalculate and reimburse nursing facilities and ~~Intermediate~~
~~Care Facilities for Individuals with Intellectual Disabilities~~
intermediate care facilities for individuals with intellectual
disabilities (ICFs/IID) from the Nursing Facility Quality of Care
Fund beginning October 1, 2000, the average actual, audited costs
reflected in previously submitted cost reports for the cost-
reporting period that began July 1, 1998, and ended June 30, 1999,

1 inflated by the federally published inflationary factors for the two
2 (2) years appropriate to reflect present-day costs at the midpoint
3 of the July 1, 2000, through June 30, 2001, rate year.

4 1. The recalculations provided for in this subsection shall be
5 consistent for both nursing facilities and ~~Intermediate Care~~
6 ~~Facilities for Individuals with Intellectual Disabilities~~
7 intermediate care facilities for individuals with intellectual
8 disabilities (ICFs/IID).

9 2. The recalculated reimbursement rate shall be implemented
10 September 1, 2000.

11 B. 1. From September 1, 2000, through August 31, 2001, all
12 nursing facilities subject to the Nursing Home Care Act, in addition
13 to other state and federal requirements related to the staffing of
14 nursing facilities, shall maintain the following minimum direct-
15 care-staff-to-resident ratios:

- 16 a. from 7:00 a.m. to 3:00 p.m., one direct-care staff to
17 every eight residents, or major fraction thereof,
- 18 b. from 3:00 p.m. to 11:00 p.m., one direct-care staff to
19 every twelve residents, or major fraction thereof, and
- 20 c. from 11:00 p.m. to 7:00 a.m., one direct-care staff to
21 every seventeen residents, or major fraction thereof.

22 2. From September 1, 2001, through August 31, 2003, nursing
23 facilities subject to the Nursing Home Care Act and ~~Intermediate~~
24 ~~Care Facilities for Individuals with Intellectual Disabilities~~

1 intermediate care facilities for individuals with intellectual
2 disabilities (ICFs/IID) with seventeen or more beds shall maintain,
3 in addition to other state and federal requirements related to the
4 staffing of nursing facilities, the following minimum direct-care-
5 staff-to-resident ratios:

- 6 a. from 7:00 a.m. to 3:00 p.m., one direct-care staff to
7 every seven residents, or major fraction thereof,
- 8 b. from 3:00 p.m. to 11:00 p.m., one direct-care staff to
9 every ten residents, or major fraction thereof, and
- 10 c. from 11:00 p.m. to 7:00 a.m., one direct-care staff to
11 every seventeen residents, or major fraction thereof.

12 3. On and after October 1, 2019, nursing facilities subject to
13 the Nursing Home Care Act and ~~Intermediate Care Facilities for~~
14 ~~Individuals with Intellectual Disabilities~~ intermediate care
15 facilities for individuals with intellectual disabilities (ICFs/IID)
16 with seventeen or more beds shall maintain, in addition to other
17 state and federal requirements related to the staffing of nursing
18 facilities, the following minimum direct-care-staff-to-resident
19 ratios:

- 20 a. from 7:00 a.m. to 3:00 p.m., one direct-care staff to
21 every six residents, or major fraction thereof,
- 22 b. from 3:00 p.m. to 11:00 p.m., one direct-care staff to
23 every eight residents, or major fraction thereof, and
24

1 c. from 11:00 p.m. to 7:00 a.m., one direct-care staff to
2 every fifteen residents, or major fraction thereof.

3 4. Effective immediately, facilities shall have the option of
4 varying the starting times for the eight-hour shifts by one (1) hour
5 before or one (1) hour after the times designated in this section
6 without overlapping shifts.

7 5. a. On and after January 1, 2020, a facility may implement
8 twenty-four-hour-based staff scheduling; provided,
9 however, such facility shall continue to maintain a
10 direct-care service rate of at least two and ~~nine~~
11 ~~tenths~~ nine-tenths (2.9) hours of direct-care service
12 per resident per day, the same to be calculated based
13 on average direct care staff maintained over a twenty-
14 four-hour period.

15 b. At no time shall direct-care staffing ratios in a
16 facility with twenty-four-hour-based staff-scheduling
17 privileges fall below one direct-care staff to every
18 fifteen residents or major fraction thereof, and at
19 least two direct-care staff shall be on duty and awake
20 at all times.

21 c. As used in this paragraph, ~~"twenty-four-hour-based-~~
22 ~~scheduling"~~ "twenty-four-hour-based staff scheduling"
23 means maintaining:
24

- (1) a direct-care-staff-to-resident ratio based on overall hours of direct-care service per resident per day rate of not less than ~~two and ninety one hundredths (2.90)~~ two and nine-tenths (2.9) hours per day,
- (2) a direct-care-staff-to-resident ratio of at least one direct-care staff person on duty to every fifteen residents or major fraction thereof at all times, and
- (3) at least two direct-care staff persons on duty and awake at all times.

6. a. On and after January 1, 2004, the State Department of Health shall require a facility to maintain the shift-based, staff-to-resident ratios provided in paragraph 3 of this subsection if the facility has been determined by the Department to be deficient with regard to:

- (1) the provisions of paragraph 3 of this subsection,
- (2) fraudulent reporting of staffing on the Quality of Care Report, or
- (3) a complaint or survey investigation that has determined substandard quality of care as a result of insufficient staffing.

1 b. The Department shall require a facility described in
2 subparagraph a of this paragraph to achieve and
3 maintain the shift-based, staff-to-resident ratios
4 provided in paragraph 3 of this subsection for a
5 minimum of three (3) months before being considered
6 eligible to implement twenty-four-hour-based staff
7 scheduling as defined in subparagraph c of paragraph 5
8 of this subsection.

9 c. Upon a subsequent determination by the Department that
10 the facility has achieved and maintained for at least
11 three (3) months the shift-based, staff-to-resident
12 ratios described in paragraph 3 of this subsection,
13 and has corrected any deficiency described in
14 subparagraph a of this paragraph, the Department shall
15 notify the facility of its eligibility to implement
16 twenty-four-hour-based staff-scheduling privileges.

17 7. a. For facilities that utilize twenty-four-hour-based
18 staff-scheduling privileges, the Department shall
19 monitor and evaluate facility compliance with the
20 twenty-four-hour-based staff-scheduling staffing
21 provisions of paragraph 5 of this subsection through
22 reviews of monthly staffing reports, results of
23 complaint investigations and inspections.

1 b. If the Department identifies any quality-of-care
2 problems related to insufficient staffing in such
3 facility, the Department shall issue a directed plan
4 of correction to the facility found to be out of
5 compliance with the provisions of this subsection.

6 c. In a directed plan of correction, the Department shall
7 require a facility described in subparagraph b of this
8 paragraph to maintain shift-based, staff-to-resident
9 ratios for the following periods of time:

10 (1) the first determination shall require that shift-
11 based, staff-to-resident ratios be maintained
12 until full compliance is achieved,

13 (2) the second determination within a two-year period
14 shall require that shift-based, staff-to-resident
15 ratios be maintained for a minimum period of
16 twelve (12) months, and

17 (3) the third determination within a two-year period
18 shall require that shift-based, staff-to-resident
19 ratios be maintained. The facility may apply for
20 permission to use twenty-four-hour staffing
21 methodology after two (2) years.

22 C. Effective September 1, 2002, facilities shall post the names
23 and titles of direct-care staff on duty each day in a conspicuous
24 place, including the name and title of the supervising nurse.

1 D. The State Commissioner of Health shall promulgate rules
2 prescribing staffing requirements for ~~Intermediate Care Facilities~~
3 ~~for Individuals with Intellectual Disabilities~~ intermediate care
4 facilities for individuals with intellectual disabilities serving
5 six or fewer clients (ICFs/IID-6) and for ~~Intermediate Care~~
6 ~~Facilities for Individuals with Intellectual Disabilities~~
7 intermediate care facilities for individuals with intellectual
8 disabilities serving sixteen or fewer clients (ICFs/IID-16).

9 E. Facilities shall have the right to appeal and to the
10 informal dispute resolution process with regard to penalties and
11 sanctions imposed due to staffing noncompliance.

12 F. 1. When the state Medicaid program reimbursement rate
13 reflects the sum of Ninety-four Dollars and eleven cents (\$94.11),
14 plus the increases in actual audited costs over and above the actual
15 audited costs reflected in the cost reports submitted for the most
16 current cost-reporting period and the costs estimated by the
17 Oklahoma Health Care Authority to increase the direct-care, flexible
18 staff-scheduling staffing level from two and eighty-six one-
19 hundredths (2.86) hours per day per occupied bed to three and two-
20 tenths (3.2) hours per day per occupied bed, all nursing facilities
21 subject to the provisions of the Nursing Home Care Act and
22 ~~Intermediate Care Facilities for Individuals with Intellectual~~
23 ~~Disabilities~~ intermediate care facilities for individuals with
24 intellectual disabilities (ICFs/IID) with seventeen or more beds, in

1 addition to other state and federal requirements related to the
2 staffing of nursing facilities, shall maintain direct-care, flexible
3 staff-scheduling staffing levels based on an overall three and two-
4 tenths (3.2) hours per day per occupied bed.

5 2. When the state Medicaid program reimbursement rate reflects
6 the sum of Ninety-four Dollars and eleven cents (\$94.11), plus the
7 increases in actual audited costs over and above the actual audited
8 costs reflected in the cost reports submitted for the most current
9 cost-reporting period and the costs estimated by the Oklahoma Health
10 Care Authority to increase the direct-care flexible staff-scheduling
11 staffing level from three and two-tenths (3.2) hours per day per
12 occupied bed to three and eight-tenths (3.8) hours per day per
13 occupied bed, all nursing facilities subject to the provisions of
14 the Nursing Home Care Act and ~~Intermediate Care Facilities for~~
15 ~~Individuals with Intellectual Disabilities~~ intermediate care
16 facilities for individuals with intellectual disabilities (ICFs/IID)
17 with seventeen or more beds, in addition to other state and federal
18 requirements related to the staffing of nursing facilities, shall
19 maintain direct-care, flexible staff-scheduling staffing levels
20 based on an overall three and eight-tenths (3.8) hours per day per
21 occupied bed.

22 3. When the state Medicaid program reimbursement rate reflects
23 the sum of Ninety-four Dollars and eleven cents (\$94.11), plus the
24 increases in actual audited costs over and above the actual audited

costs reflected in the cost reports submitted for the most current cost-reporting period and the costs estimated by the Oklahoma Health Care Authority to increase the direct-care, flexible staff-scheduling staffing level from three and eight-tenths (3.8) hours per day per occupied bed to four and one-tenth (4.1) hours per day per occupied bed, all nursing facilities subject to the provisions of the Nursing Home Care Act and ~~Intermediate Care Facilities for Individuals with Intellectual Disabilities~~ intermediate care facilities for individuals with intellectual disabilities (ICFs/IID) with seventeen or more beds, in addition to other state and federal requirements related to the staffing of nursing facilities, shall maintain direct-care, flexible staff-scheduling staffing levels based on an overall four and one-tenth (4.1) hours per day per occupied bed.

4. The Commissioner shall promulgate rules for shift-based, staff-to-resident ratios for noncompliant facilities denoting the incremental increases reflected in direct-care, flexible staff-scheduling staffing levels.

5. In the event that the state Medicaid program reimbursement rate for facilities subject to the Nursing Home Care Act, ~~and Intermediate Care Facilities for Individuals with Intellectual Disabilities~~ intermediate care facilities for individuals with intellectual disabilities (ICFs/IID) having seventeen or more beds is reduced below actual audited costs, the requirements for staffing

ratio levels shall be adjusted to the appropriate levels provided in paragraphs 1 through 4 of this subsection.

G. For purposes of this ~~subsection~~ section:

1. "Direct-care staff" means any nursing or therapy staff who provides direct, hands-on care to residents in a nursing facility;

2. Prior to September 1, 2003, activity and social services staff who are not providing direct, hands-on care to residents may be included in the direct-care-staff-to-resident ratio in any shift.

On and after September 1, 2003, such persons shall not be included in the direct-care-staff-to-resident ratio, regardless of their licensure or certification status; and

3. The administrator shall not be counted in the direct-care-staff-to-resident ratio regardless of the administrator's licensure or certification status.

H. 1. The Oklahoma Health Care Authority shall require all nursing facilities subject to the provisions of the Nursing Home Care Act and ~~Intermediate Care Facilities for Individuals with Intellectual Disabilities~~ intermediate care facilities for individuals with intellectual disabilities (ICFs/IID) with seventeen or more beds to submit a monthly report on staffing ratios on a form that the Authority shall develop.

2. The report shall document the extent to which such facilities are meeting or are failing to meet the minimum direct-

1 care-staff-to-resident ratios specified by this section. Such
2 report shall be available to the public upon request.

3 3. The Authority may assess administrative penalties for the
4 failure of any facility to submit the report as required by the
5 Authority. Provided, however:

6 a. administrative penalties shall not accrue until the
7 Authority notifies the facility in writing that the
8 report was not timely submitted as required, and

9 b. a minimum of a one-day penalty shall be assessed in
10 all instances.

11 4. Administrative penalties shall not be assessed for
12 computational errors made in preparing the report.

13 5. Monies collected from administrative penalties shall be
14 deposited in the Nursing Facility Quality of Care Fund established
15 in Section 2002 of Title 56 of the Oklahoma Statutes and utilized
16 for the purposes specified in ~~the Oklahoma Healthcare Initiative Act~~
17 such section.

18 I. 1. All entities regulated by this state that provide long-
19 term care services shall utilize a single assessment tool to
20 determine client services needs. The tool shall be developed by the
21 Oklahoma Health Care Authority in consultation with the State
22 Department of Health.

1 2. a. The Oklahoma Nursing Facility Funding Advisory
2 Committee is hereby created and shall consist of the
3 following:

4 (1) four members selected by ~~the Oklahoma Association~~
5 ~~of Health Care Providers~~ Care Providers Oklahoma
6 or its successor organization,

7 (2) three members selected by ~~the Oklahoma~~
8 ~~Association of Homes and Services for the Aging~~
9 LeadingAge Oklahoma or its successor
10 organization, and

11 (3) two members selected by the ~~State Council on~~
12 ~~Aging~~ State Council on Aging and Adult Protective
13 Services.

14 The ~~Chair~~ chair shall be elected by the committee. No
15 state employees may be appointed to serve.

16 b. The purpose of the advisory committee ~~will~~ shall be to
17 develop a new methodology for calculating state
18 Medicaid program reimbursements to nursing facilities
19 by implementing facility-specific rates based on
20 expenditures relating to direct care staffing.
21 No nursing home ~~will~~ shall receive less than the
22 current rate at the time of implementation of
23 facility-specific rates pursuant to this subparagraph.

1 c. The advisory committee shall be staffed and advised by
2 the Oklahoma Health Care Authority.

3 d. The new methodology ~~will~~ shall be submitted for
4 approval to the ~~Board of the~~ Oklahoma Health Care
5 Authority Board by January 15, 2005, and shall be
6 finalized by July 1, 2005. The new methodology ~~will~~
7 shall apply only to new funds that become available
8 for Medicaid nursing facility reimbursement after the
9 methodology of this paragraph has been finalized.
10 Existing funds paid to nursing homes ~~will~~ shall not be
11 subject to the methodology of this paragraph. The
12 methodology as outlined in this paragraph ~~will~~ shall
13 only be applied to any new funding for nursing
14 facilities appropriated above and beyond the funding
15 amounts effective on January 15, 2005.

16 e. The new methodology shall divide the payment into two
17 components:

18 (1) direct care which includes allowable costs for
19 ~~registered nurses~~ Registered Nurses, ~~licensed~~
20 ~~practical nurses~~ Licensed Practical Nurses,
21 ~~certified medication aides~~ Certified Medication
22 Aides and ~~certified nurse aides~~ Certified Nurse
23 Aides. The direct care component of the rate
24 shall be a facility-specific rate, directly

1 related to each facility's actual expenditures on
2 direct care, and

3 (2) other costs.

4 f. The Oklahoma Health Care Authority, in calculating the
5 base year prospective direct care rate component,
6 shall use the following criteria:

7 (1) to construct an array of facility per diem
8 allowable expenditures on direct care, the
9 Authority shall use the most recent data
10 available. The limit on this array shall be no
11 less than the ninetieth percentile,

12 (2) each facility's direct care base-year component
13 of the rate shall be the lesser of the facility's
14 allowable expenditures on direct care or the
15 limit,

16 (3) other rate components shall be determined by the
17 Oklahoma Nursing Facility Funding Advisory
18 Committee or the Authority in accordance with
19 federal regulations and requirements,

20 (4) prior to July 1, 2020, the Authority shall seek
21 federal approval to calculate the upper payment
22 limit under the authority of ~~CMS~~ the Centers for
23 Medicare and Medicaid Services (CMS) utilizing
24 the Medicare equivalent payment rate, and

1 (5) if Medicaid payment rates to providers are
2 adjusted, nursing home rates and ~~Intermediate~~
3 ~~Care Facilities for Individuals with Intellectual~~
4 ~~Disabilities~~ intermediate care facilities for
5 individuals with intellectual disabilities
6 (ICFs/IID) rates shall not be adjusted less
7 favorably than the average percentage-rate
8 reduction or increase applicable to the majority
9 of other provider groups.

- 10 g. (1) Effective October 1, 2019, if sufficient funding
11 is appropriated for a rate increase, a new
12 average rate for nursing facilities shall be
13 established. The rate shall be equal to the
14 statewide average cost as derived from audited
15 cost reports for SFY 2018, ending June 30, 2018,
16 after adjustment for inflation. After such new
17 average rate has been established, the facility
18 specific reimbursement rate shall be as follows:
- 19 (a) amounts up to the existing base rate amount
20 shall continue to be distributed as a part
21 of the base rate in accordance with the
22 existing Medicaid State Plan, and
 - 23 (b) to the extent the new rate exceeds the rate
24 effective before ~~the effective date of this~~

1 ~~at~~ October 1, 2019, fifty percent (50%) of
2 the resulting increase on October 1, 2019,
3 shall be allocated toward an increase of the
4 existing base reimbursement rate and
5 distributed accordingly. The remaining
6 fifty percent (50%) of the increase shall be
7 allocated in accordance with the currently
8 approved 70/30 reimbursement rate
9 methodology as outlined in the existing
10 Medicaid State Plan.

11 (2) Any subsequent rate increases, as determined
12 based on the provisions set forth in this
13 subparagraph, shall be allocated in accordance
14 with the currently approved 70/30 reimbursement
15 rate methodology. The rate shall not exceed the
16 upper payment limit established by the Medicare
17 rate equivalent established by the federal CMS.

18 h. Effective October 1, 2019, in coordination with the
19 rate adjustments identified in the preceding section,
20 a portion of the funds shall be utilized as follows:

21 (1) effective October 1, 2019, the Oklahoma Health
22 Care Authority shall increase the personal needs
23 allowance for residents of nursing homes and
24 ~~Intermediate Care Facilities for Individuals with~~

~~Intellectual Disabilities~~ intermediate care facilities for individuals with intellectual disabilities (ICFs/IID) from Fifty Dollars (\$50.00) per month to Seventy-five Dollars (\$75.00) per month per resident. The increase shall be funded by Medicaid nursing home providers, by way of a reduction of eighty-two cents (\$0.82) per day deducted from the base rate. Any additional cost shall be funded by the Nursing Facility Quality of Care Fund, and

(2) effective January 1, 2020, all clinical employees working in a licensed nursing facility shall be required to receive at least four (4) hours annually of Alzheimer's or dementia training, to be provided and paid for by the facilities.

i. (1) Upon the effective date of this act, the Authority shall prepare and submit a Medicaid state plan amendment to the federal Centers for Medicare and Medicaid Services and seek any other federal approval necessary to implement the provisions of this subparagraph.

(2) Upon receipt of any necessary federal approval, and subject to the availability of funds, the Authority shall establish an add-on rate for

1 nursing facilities participating in the state
2 Medicaid program that engage in multigenerational
3 activities as defined in 42 U.S.C., Section
4 3032f(h) .

5 (3) The Oklahoma Health Care Authority Board shall
6 promulgate rules to implement the provisions of
7 this subparagraph.

8 3. The Department of Human Services shall expand its statewide
9 toll-free, ~~Senior-Info-Line~~ Senior Info-line for senior citizen
10 services to include assistance with or information on long-term care
11 services in this state.

12 4. The Oklahoma Health Care Authority shall develop a nursing
13 facility cost-reporting system that reflects the most current costs
14 experienced by nursing and specialized facilities. The Oklahoma
15 Health Care Authority shall utilize the most current cost report
16 data to estimate costs in determining daily per diem rates.

17 5. The Oklahoma Health Care Authority shall provide access to
18 the detailed Medicaid payment audit adjustments and implement an
19 appeal process for disputed payment audit adjustments to the
20 provider. Additionally, the Oklahoma Health Care Authority shall
21 make sufficient revisions to the nursing facility cost reporting
22 forms and electronic data input system so as to clarify what
23 expenses are allowable and appropriate for inclusion in cost
24 calculations.

J. 1. When the state Medicaid program reimbursement rate reflects the sum of Ninety-four Dollars and eleven cents (\$94.11), plus the increases in actual audited costs, over and above the actual audited costs reflected in the cost reports submitted for the most current cost-reporting period, and the direct-care, flexible staff-scheduling staffing level has been prospectively funded at four and one-tenth (4.1) hours per day per occupied bed, the Authority may apportion funds for the implementation of the provisions of this section.

2. The Authority shall make application to the United States Centers for Medicare and Medicaid ~~Service~~ Services for a waiver of the uniform requirement on health-care-related taxes as permitted by ~~Section 433.72 of 42 C.F.R.,~~ Section 433.72.

3. Upon approval of the waiver, the Authority shall develop a program to implement the provisions of the waiver as it relates to all nursing facilities.

SECTION 2. This act shall become effective July 1, 2026.

SECTION 3. It being immediately necessary for the preservation of the public peace, health or safety, an emergency is hereby declared to exist, by reason whereof this act shall take effect and be in full force from and after its passage and approval.

60-2-3985 DC 4/29/2026 10:48:02 AM